



Gender: Male

DOB:

Race: White

Report Date:

Pathology Report:

ICD-O-3

carcinoma urothelial, nos 8120/3

CQCF Site: bladder, wall, lateral C67.3

Path: bladder, nos C67.9

Surgical Pathology Report

hw
9/29/12

FINAL PATHOLOGIC DIAGNOSIS

A. Left pelvic lymph nodes; dissection:

- Seven lymph nodes, no tumor (0/7).

B. Right pelvic lymph nodes; dissection:

- Nine lymph nodes, no tumor (0/9).

C. Urinary bladder, prostate, and seminal vesicles; cystoprostatectomy:

- Bladder with invasive urothelial carcinoma with squamous differentiation,
see pathologic parameters.

< 10% squamous per TSS. 9/7/12 hw

- Urothelial carcinoma invades thru the bladder wall, into the perivesical fat
and left seminal vesicle.

- Urothelial carcinoma is <0.1 cm away from the posterior margin.

- Adenocarcinoma of the prostate, see pathologic parameters.

D. Right common iliac lymph nodes; dissection:

- Six lymph nodes, no tumor (0/6).

E. Left common iliac lymph nodes; dissection:

- Seven lymph nodes, no tumor (0/7).

F. Presacral lymph nodes; dissection:

- Seven lymph nodes, no tumor (0/7).

G. Left distal ureter; biopsy:

- Portion of ureter, negative for tumor.

H. Right distal ureter; biopsy:

- Portion of ureter, negative for tumor.

Criteria	Yes	No	
Diagnosis Discrepancy		X	hw
Primary Tumor Site Discrepancy		X	
IIPAA Discrepancy		X	9/7/12
Prior Malignancy History		X	
Local/Synchronous Primary Noted	X		
Case is (circle):	QUALIFIED	DISQUALIFIED	
Reviewer Initials	hw		
Date Reviewed	8/17/12		

Urothelial Carcinoma Pathologic Parameters

1. Tumor type: Urothelial carcinoma, with squamous differentiation.

2. Grade of tumor: High grade.
3. Depth of invasion: Extravesicular soft tissues and left seminal vesicle.
4. Tumor distribution: Solitary (bladder left posterior wall, lateral wall and trigone), 4.7 cm.
5. Ureteral margins: Negative for tumor.
6. Distal urethral margin: Negative for tumor.
7. Soft tissue margin or serosa: Negative for tumor (tumor is <0.1 cm from the inked margin).
8. Lymph nodes: Negative for tumor (0/36).
9. pTNM: pT4,N0,MX. (pT4 for seminal vesicle involvement).

Radical Prostatectomy Pathologic Parameters

1. Gleason Score: 3+4=7.
2. Perineural Invasion: Present.
3. Tumor Location: Peripheral zone - right and left.
4. Tumor Volume Estimate: 20%.
5. High-grade P.I.N.: Multifocal.
6. Seminal Vesicles: Negative for tumor.
7. Extraprostatic extension: Present.
8. Peripheral Margin: Negative for tumor.
9. Distal (apical) Margin: Negative for tumor.
10. Proximal (basilar) Margin: Negative for tumor.
11. Regional Lymph Nodes (right and left): Negative for tumor (0/36).
12. pTNM: pT3a,N0,MX.

this Checklist utilizes the 7th edition TNM staging system for prostatectomy of the American Joint Committee on Cancer (AJCC) and the International Union Against Cancer (UICC).

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[] MD

Interpretation performed by the Attending Pathologist and reviewed with the Resident/Fellow, [], M.D.

Electronically Signed Out by [] MD

Clinical History:

The patient is a -year-old male with a history of bladder cancer. He undergoes cystectomy and ileal conduit.

Specimens Received:

- A: Left pelvic lymph nodes
- B: Right pelvic lymph nodes
- C: Bladder, prostate, seminal vesicles, and vas deferens

D: Right common iliac
E: Left common iliac
F: Pre-sacral
G: Distal left ureter
H: Distal right ureter

Gross Description:

The specimens are received in eight containers each labeled with the patient's name and medical record number.

A. The first container is additionally identified as, "left pelvic lymph nodes". Received fresh and placed in formalin is a 5.0 x 3.5 x 2.5 cm aggregate of lobulated yellow fibrofatty tissue. The specimen is dissected for lymph nodes to reveal multiple is as apparent lymph nodes ranging from 0.3-3.2 cm in greatest dimension. All apparent lymph nodes are submitted as follows:

A1: 4 lymph nodes

A2-6: One lymph node in each cassette, bisected

B. The second container is additionally identified as, "right pelvic lymph nodes". Received fresh and placed in formalin is a 5.0 x 3.5 x 2.5 cm aggregate of lobulated yellow fibrofatty tissue. The specimen is dissected for lymph nodes to reveal multiple apparent lymph nodes ranging from 0.2-3.5 cm in greatest dimension. All apparent lymph nodes are submitted as follows:

B1-2: 3 lymph nodes in each cassette

B3-4: 1 lymph node in each cassette, bisected

B5-6: 1 lymph node, serially sectioned

C. The third container is additionally identified as, "bladder, prostate, seminal vesicles and vas deferens". Received fresh and placed in formalin is a 408.2 g, 19 x 5.5 x 3.5 cm specimen consisting of a 10 x 9 x 3 cm bladder with an attached 3.8 x 2.9 x 2 cm prostate. The left ureter is 2 cm in length by 0.7 cm in diameter. The left ureter is not probe patent to the trigone. The right ureter is 1.5 cm in length by 0.4 cm in diameter. The right ureter is probe patent to the trigone. There is a 0.4 cm in length by 0.7 cm in diameter urethral stump which is patent. The right seminal vesicle is 3.2 x 0.8 x 0.5 cm and the right 5 is 9.5 cm in length by 0.3-0.5 cm in diameter. The left seminal vesicle and left vase are matted together along the left posterior aspect of the specimen and therefore cannot be measured. Also received are 2 detached vessels measuring 5.5 cm and 8 cm in length by 0.3 cm in diameter. The specimen is inked blue on the right and black on the left side of the prostate and the serous serosal surface of the bladder is inked black. It is then opened anteriorly to reveal a 4.7 x 4.5 cm indurated lesion which is tan-gray and ulcerated involving the left posterior, trigone and left lateral wall. This area completely involves and occludes the left ureter orifice. Gross photographs are taken. This lesion is 3.9 cm from the distal ureter margin and 0.2 cm from the right ureter orifice. The lesion is surrounded by hyperemic, erythematous mucosa. On the

posterior wall there are 2 areas with grey-brown exudate adherent to the mucosa. These areas measure 2 x 1.5 cm and 0.7 x 0.6 cm. The remaining mucosa is glistening, tan-pink and grossly unremarkable. The large lesion is serially sectioned to reveal a firm white cut surface extending to a depth of 2.1 cm, which surrounds the left seminal vesicle and vas and stretches the inked black serosal surface. Serial sectioning of the prostate reveals gritty brown material present within cystically dilated spaces involving the left side of the prostate. Representative sections are submitted as follows:

C1: Left ureter margin

C2: Right ureter margin

C3: Urethral margin

C4-C5: Apical margin of the prostate, serially sectioned

C6-C11: Representative sections of prostate

C12: Right seminal vesicle and vas

C13: Left seminal vesicle and vas

C14: Tumor from trigone

C15: Tumor, left lateral wall

C16: Tumor, posterior wall (demonstrates tumor approaching inked serosal surface)

C17: Left lateral wall, tumor surrounding seminal vesicle and vas

C18: Uninvolved right lateral wall

C19: posterior wall, small exudate/plaque

C20: posterior wall, larger plaque

C21: Uninvolved dome

C22: Uninvolved anterior wall

C23: Additional section of tumor, left lateral wall

D. The fourth container is additionally identified as, "right common iliac". Received fresh and placed in formalin is a 4.5 x 3.0 x 1.4 cm aggregate of lobulated yellow fibrofatty tissue. The specimen is dissected for lymph nodes to reveal multiple apparent lymph nodes ranging from 0.1-0.3 cm in greatest dimension. All apparent lymph nodes are submitted as follows:

D1-2: 4 lymph nodes in each cassette

D3-6: Representative lymphovascular tissue

E. The fifth container is additionally identified as, "left common iliac".

Received fresh and placed in formalin is a 3.5 x 2.5 x 1.5 cm aggregate of lobulated yellow fibrofatty tissue. The specimen is dissected for lymph nodes to reveal multiple apparent lymph nodes ranging from 0.3-0.5 cm in greatest dimension. The specimen is entirely submitted as follows:

E1-2: 2 lymph nodes in each cassette

E3-4: Remaining tissue

F. The sixth container is additionally identified as, "pre-sacral". Received fresh and placed in formalin is a 2.0 x 1.5 x 0.7 cm aggregate of lobulated yellow fibrofatty tissue. The specimen is dissected for lymph nodes to reveal

multiple apparent lymph nodes ranging from 0.2-0.3 cm in greatest dimension. The specimen is entirely submitted as follows:

F1: 1 lymph node

F2: 3 lymph nodes

F3: Remaining specimen

G. The seventh container is additionally identified as, "7. Distal left ureter, stitch is distal ". Received fresh and placed in formalin is a 3.3 cm portion of ureter with a 1.1 cm diameter. The outer surface is tan and smooth with scattered areas of hemorrhage. There is a scant amount of attached yellow adipose tissue. There is a clear suture at the distal end. The specimen is serially sectioned to reveal a wall thickness of 0.1 cm and tan unremarkable mucosa. Representative sections are submitted as follows:

G1: Distal end

G2: Proximal end

G3: Representative sections from the center of the specimen

H. The eighth container is additionally identified as, "8. Distal right ureter, stitch on non-margin side ". Received fresh and placed in formalin is a 0.1 cm portion of ureter with a 0.6 cm diameter. The outer surface is pink to tan and glistening with a 2 x 0.9 x 0.5 cm portion of attached yellow adipose tissue. There is a clear suture at the non-margin side. The remainder of the specimen is serially sectioned to reveal a pinpoint lumen and a wall thickness of 0.2 cm.

Representative sections are submitted as follows:

H1: Margin side

H2: Non-margin side

H3: Representative sections from the center of the specimen

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[] M.D.